

# Uphamol Plus Codeine Tablet



**Relieves moderate pain and fever**

## DESCRIPTION

A 12mm, round, flat, scored and yellow tablet.  
Each tablet contains Paracetamol 500mg and Codeine Phosphate 8mg.  
This product contains gelatin derived from Bovine source certified with Malaysia's Halal certification.

Route of Administration : Oral

## PHARMACODYNAMICS

The analgesic and antipyretic actions of paracetamol are similar to those of the salicylates. Analgesia is mediated peripherally and also centrally, whereas antipyresis is produced by a central action on the hypothalamic regulatory centre. Uphamol Plus Codeine Tablet also contains a small amount of codeine, an opiate with analgesic and antitussive actions, to provide additional pain relief.

## PHARMACOKINETICS

Paracetamol is rapidly and almost completely absorbed from the gastrointestinal tract following oral administration. Following oral administration of immediate release paracetamol preparation, peak plasma concentration are attained within 10 – 60 minutes. Paracetamol has been reported to have a plasma half-life of 1.25 - 3 hours. Paracetamol is rapidly and uniformly distributed into most body tissue except fat. About 10 – 25% of paracetamol in blood is bound to plasma proteins.

Codeine phosphate is well absorbed after oral administration. Codeine is distributed into milk. Codeine is metabolised mainly in the liver; the drug is excreted mainly in urine as norcodeine and free and conjugated morphine. Negligible amounts of codeine and its metabolites are found in feces.

## INDICATIONS

Uphamol Plus Codeine Tablet is indicated for the relief of painful disorders such as headache, dysmenorrhoea, conditions involving musculoskeletal pain, myalgias and neuralgia. It is also indicated as an analgesic and antipyretic in conditions accompanied by discomfort and fever, such as the common cold and viral infections. Uphamol Plus Codeine Tablet is an effective analgesic after dental work and tooth extractions.

Codeine is indicated in patients older than 12 years of age for the treatment of acute moderate pain which is not considered to be relieved by other analgesics such as paracetamol or ibuprofen (alone).

## CONTRAINDICATIONS

- Hypersensitivity to paracetamol or codeine.
- In children below the age of 12 years for the symptomatic treatment of colds due to an increased risk of developing serious and life-threatening adverse reactions.
- In all paediatric patients (0-18 years of age) who undergo tonsillectomy and/or adenoidectomy for obstructive sleep apnoea syndrome due to increased risk of developing serious and life-threatening adverse reactions.
- In women who are breastfeeding.
- In patients for whom it is known they are CYP2D6 ultra-rapid metabolisers.

## PRECAUTIONS

Codeine may be habit forming.

Use Uphamol Plus Codeine Tablet with caution in patients with acute abdominal conditions and in other special risk patients (e.g., those with head injury, increased intracranial pressure, or with Addison's Disease).

Information for Patients: Patients should be advised that this product contains codeine, which can produce sedation in some individuals.

### CYP2D6 metabolism

Codeine is metabolised by the liver enzyme CYP2D6 into morphine, its active metabolite. If a patient has a deficiency or is completely lacking this enzyme an adequate analgesic effect will not be obtained. Estimates indicate that up to 7% of the Caucasian population may have this deficiency. However, if the patient is an extensive

or ultra-rapid metaboliser there is an increased risk of developing side effects of opioid toxicity even at commonly prescribed doses. These patients convert codeine into morphine rapidly resulting in higher than expected serum morphine levels.

General symptoms of opioid toxicity include confusion, somnolence, shallow breathing, small pupils, nausea, vomiting, constipation and lack of appetite. In severe cases this may include symptoms of circulatory and respiratory depression, which may be life-threatening and very rarely fatal. Estimates of prevalence of ultra-rapid metabolisers in different populations are summarised below:

| Population        | Prevalence % |
|-------------------|--------------|
| African/Ethiopian | 29%          |
| African American  | 3.4 to 6.5%  |
| Asian             | 1.2 to 2.0%  |
| Caucasian         | 3.6 to 6.5%  |
| Greek             | 6.0%         |
| Hungarian         | 1.9%         |
| Northern European | 1.0 to 2.0%  |

### Post-operative use in children

There have been reports in the published literature that codeine given post-operatively in children after tonsillectomy and/or adenoidectomy for obstructive sleep apnoea, led to rare, but life-threatening adverse events including death. All children received doses of codeine that were within the appropriate dose range; however there was evidence that these children were either ultra-rapid or extensive metabolisers in their ability to metabolise in their ability to metabolise codeine to morphine.

### Children with compromised respiratory function

Codeine is not recommended for use in children in whom respiratory function might be compromised including neuromuscular disorders, severe cardiac or respiratory conditions, upper respiratory or lung infections, multiple trauma or extensive surgical procedures. These factors may worsen symptoms of morphine toxicity.

### Risks from Concomitant Use with Benzodiazepines

Profound sedation, respiratory depression, coma, and death may result from the concomitant use of Uphamol Plus Codeine Tablet with benzodiazepines. Observational studies have demonstrated that concomitant use of opioids and benzodiazepines increases the risk of drug-related mortality compared to use of opioids alone. Because of these risks, reserve concomitant prescribing of these drugs for use in patients for whom alternative treatment options are inadequate.

If the decision is made to newly prescribe a benzodiazepine and an opioid together, prescribe the lowest effective dosages and minimum durations of concomitant use.

If the decision is made to prescribe a benzodiazepine in a patient already receiving an opioid, prescribe a lower initial dose of the benzodiazepine than indicated in the absence of an opioid, and titrate based on clinical response.

If the decision is made to prescribe an opioid in a patient already taking a benzodiazepine, prescribe a lower initial dose of the opioid, and titrate based on clinical response. Follow patients closely for signs and symptoms of respiratory depression and sedation. Advise both patients and caregivers about the risks of respiratory depression and sedation when Uphamol Plus Codeine Tablet is used with benzodiazepines. Advise patients not to drive or operate heavy machinery until the effects of concomitant use of the benzodiazepine have been determined. Screen patients for risk of substance use disorders, including opioid abuse and misuse, and warn them of the risk for overdose and death associated with the use of benzodiazepines (See Drug Interactions).

### Serotonin Syndrome with Concomitant Use of Serotonergic Drugs

Cases of serotonin syndrome, a potentially life-threatening condition, have been reported during concurrent use of Uphamol Plus Codeine with serotonergic drugs. (See Drug Interaction). This may occur within the recommended dosage range. Serotonin syndrome symptoms may include mental-status changes (e.g. agitation, hallucinations, coma), autonomic instability (e.g. tachycardia, labile blood pressure, hyperthermia), neuromuscular aberrations (e.g. hyperreflexia, incoordination) and/or gastrointestinal symptoms (e.g. nausea, vomiting, diarrhoea) and can be fatal (See Drug Interaction). The onset of symptoms generally occurs within several hours to a few days of concomitant use, but may occur later than that. Discontinue Uphamol Plus Codeine if serotonin syndrome is suspected.

### Adrenal Insufficiency

Cases of adrenal insufficiency have been reported with opioid use, more often following greater than one month of use. Presentation of adrenal insufficiency may

UPHAMOL PLUS CODEINE TABLET

170mm(w) x 198mm (h)



Black 100%

include nonspecific symptoms and signs including nausea, vomiting, decreased appetite, fatigue, weakness, dizziness and low blood pressure. If adrenal insufficiency is suspected, confirm the diagnosis with diagnostic testing as soon as possible. If adrenal insufficiency is diagnosed, treat with physiologic replacement dosing of corticosteroids. Wean the patient off of the opioid to allow adrenal function to recover and continue corticosteroid treatment until adrenal function recovers. Other opioids may be tried as some cases reported use of a different opioid without the recurrence of adrenal insufficiency. The information available does not identify any particular opioids as being more likely to be associated with adrenal insufficiency.

#### Sexual Function/Reproduction

Long term use of opioids may be associated with decreased sex hormone levels and symptoms such as low libido, erectile dysfunction, or infertility (See Postmarketing Experience)

This preparation contains PARACETAMOL. Do not take any other PARACETAMOL containing medicines at the same time

#### **WARNING**

**Allergy alert:** Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

#### **INTERACTIONS**

Patients receiving other CNS depressant, including alcohol, may exhibit additive CNS effects.

##### Benzodiazepines

Due to additive pharmacologic effect, the concomitant use of opioids with benzodiazepines increase the risk of respiratory depression, profound sedation, coma and death. The concomitant use of opioids and benzodiazepines increases the risk of respiratory depression because of actions at different receptor sites in the central nervous system that control respiration. Opioids interact primarily at  $\mu$ -receptors, and benzodiazepines interact at GABA<sub>A</sub> sites. When opioids and benzodiazepines are combined, the potential for benzodiazepines to significantly worsen opioid-related respiratory depression exists.

Reserve concomitant prescribing of these drugs for use in patients for whom alternative treatment options are inadequate (see Warnings and Precautions).

Limit dosage and duration of concomitant use of benzodiazepines and opioids, and follow patients closely for respiratory depression and sedation.

##### Serotonergic Drugs

The concomitant use of opioids with other drugs that affect the serotonergic neurotransmitter system has resulted in serotonin syndrome. If concomitant use is warranted, carefully observe the patient, particularly during treatment initiation and dose adjustment. Discontinue Uphamol Plus Codeine if serotonin syndrome is suspected. Examples of serotonergic drugs are selective serotonin reuptake inhibitors (SSRIs), serotonin and norepinephrine reuptake inhibitors (SNRIs), tricyclic antidepressants (TCAs), triptans, 5-HT<sub>3</sub> receptor antagonists, drugs that affect the serotonin neurotransmitter system (e.g. mirtazapine, trazodone, tramadol), monoamine oxidase (MAO) inhibitors (those intended to treat psychiatric disorders and also others, such as linezolid and intravenous methylene blue) (see Warnings and Precautions).

#### **PREGNANCY AND LACTATION**

##### Pregnancy:

Careful consideration should be given before prescribing the product for pregnant patients. Opioid analgesics may depress neonatal respiration and cause withdrawal effects in neonates of pregnant mothers.

As a precautionary measure, use of Uphamol Plus Codeine Tablet should be avoided during the third trimester of pregnancy and during labor.

##### Breast-feeding:

Uphamol Plus Codeine Tablet is contraindicated in women during breastfeeding. At normal therapeutic doses codeine and its active metabolite may be present in breast milk at very low doses and is unlikely to adversely affect the breast fed infant. However, if the patient in an ultra-rapid metabolite, morphine, may be present in breast milk and on very rare occasions may result in symptoms of opioid toxicity in the infant, which may be fatal.

#### **SIDE EFFECTS**

The most frequently observed adverse reactions to codeine include light headedness, dizziness, sedation, nausea and vomiting, but these generally occur with a dose larger than that in Uphamol Plus Codeine Tablet. When taken in recommended doses, paracetamol is usually free from side effects. Skin reactions, such as urticaria, have been described rarely. Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/Toxic Epidermal Necrolysis have been reported.

##### Postmarketing Experience

Serotonin syndrome (see Warnings and Precautions)  
Adrenal insufficiency (see Warnings and Precautions)

Androgen deficiency: Cases of androgen deficiency have occurred with chronic use of opioids. Chronic use of opioids may influence the hypothalamic-pituitary-gonadal axis, leading to androgen deficiency that may manifest as low libido, impotence, erectile dysfunction, amenorrhea, or infertility. The causal role of opioids in the clinical syndromes of hypogonadism is unknown because the various medical, physical, lifestyle, and physiological stressors that may influence gonadal hormone levels have not been adequately controlled for in studies conducted to date. Patients presenting with symptoms of androgen deficiency should undergo laboratory evaluation.

Infertility: Chronic use of opioids may caused reduced fertility in females and males of reproductive potential. It is not known whether these effects on fertility are reversible.

#### **DOSAGE**

##### **Adult and children above 12 years:**

1 to 2 tablets every 4 to 6 hours (not more than 6 tablets in one day) or as directed by your medical practitioner.

##### **Children aged less than 12 years:**

Codeine should not be used in children below the age of 12 years because of the risk of opioid toxicity due to the variable and unpredictable metabolism of codeine to morphine.

Uphamol Plus Codeine Tablet is contraindicated in children below the age of 12 years for the symptomatic treatment of cold.

##### **Children aged 12 years to 18 years:**

Uphamol Plus Codeine Tablet is not recommended for use in children aged 12 years to 18 years with compromised respiratory function.

#### **OVERDOSAGE**

##### Symptoms:

Vomiting, mild nausea, gastrointestinal upset, hypertension, anorexia and diarrhoea.

Treatment (for medical practitioner's information):

- Empty the stomach via induction of emesis or gastric lavage.
- Acetylcysteine administration is recommended to be instituted immediately after ingestion of an overdose has been reported without waiting for the results of urine and plasma level tests. Acetylcysteine is most effective if treatment is started within 10 to 12 hours after the ingestion of overdose, however, it may be of some benefit if treatment is started within 24 hours. For oral administration, the recommended adult dose of acetylcysteine is 140mg/kg initially, then 70 mg/kg every 4 hours for 17 doses. Each dose should be diluted to a 5% solution of drinking fluid prior to administration because of its unpleasant odour and its sclerosing properties. Any dose vomited within 1 hour of administration must be repeated. If necessary, the antidote may be given via duodenal intubation. Plasma acetaminophen concentration should be determined at least 4 hours following the ingestion of overdose in order to ascertain the status of patient. Renal and cardiac function should be monitored and appropriate therapy instituted if necessary.

Up-to-date information on treatment of overdose can be obtained from The National Poison Centre, Universiti Sains Malaysia

#### **PACKING**

Blister pack: 10's, 10x10's, 18x10's, 50x10's and 100x10's

Not all pack sizes are locally available

#### **STORAGE CONDITIONS AND SHELF LIFE**

|                                            |
|--------------------------------------------|
| Store in a dry place below 30°C.           |
| Protect from light.                        |
| Keep out of reach of children.             |
| Jauhi daripada kanak-kanak.                |
| Shelf life: please refer to outer package. |

Product Registration Holder:

**Duopharma Manufacturing (Bangi) Sdn. Bhd.**

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1500008673 N.1 Revision

Date: June 2019

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